

50 providers · 4 months on Abridge · 65% utilization · 35,750 documented encounters

EXECUTIVE SUMMARY

In 4 months, your 50 providers have generated \$118K in measurable annual value. With expansion to 150 providers at 80% utilization, projected value reaches \$501K.

\$118K

ANNUAL VALUE CREATED

Defensible hard dollars

\$2K

VALUE / PROVIDER

Based on 50 providers

+\$383K

EXPANSION POTENTIAL

At 150 providers

DEPLOYMENT MATURITY

Utilization Rate

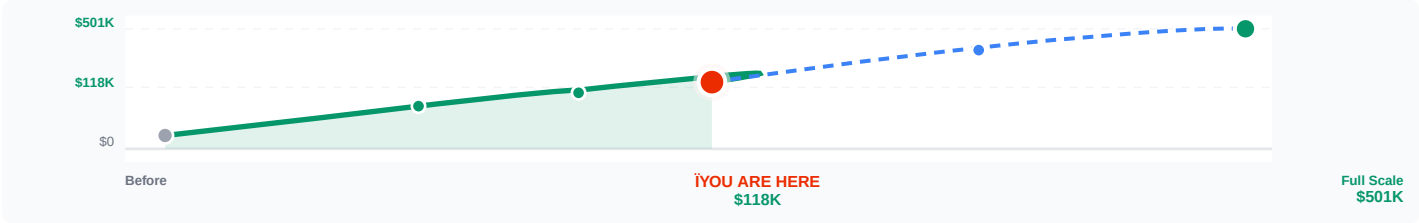
65%

Developing

VALUE INDICATORS

wRVU Per Encounter

YOUR VALUE JOURNEY



VALUE BREAKDOWN

Tier 1: Hard Value

Directly measurable financial impact

\$118K

wRVU Per Encounter

5,750 encounters × \$33/wRVU × 50% attribution

\$118K

Tier 2: Efficiency Gains

Measured improvements—not yet converted to dollars

Time Saved

2,383 hours/year

Chart Closure Improvement

+5%

**KEY TAKEAWAY** Your 50 providers are generating \$2K each annually. At 65% utilization, there's still significant room to grow—expansion to 150 providers at 80% unlocks an additional \$383K.

●

WRVU PER ENCOUNTER

Revenue capture improvement

\$118K  
annual value

BEFORE ABRIDGE

2.30  
wRVU/enc

→

AFTER ABRIDGE

2.50  
wRVU/enc

=

+0.20  
9% lift

BENCHMARK COMPARISON

Typical range: 3-7% lift

● Above typical range

VALIDATION RECOMMENDED

Your results exceed our typical range. This isn't necessarily wrong—some organizations do see higher lift, especially if baseline documentation was sparse. But we recommend confirming: Was baseline measured the same way as current? Were there other coding or documentation initiatives during this period? Is the provider population consistent between periods?

VALUE CALCULATION

+0.20 wRVU/enc × 35,750 encounters × \$33/wRVU × 50% attribution = \$118K

We use Medicare's \$33 conversion factor and 50% attribution to Abridge. If your payer mix is commercial-heavy (where conversion factors run \$45-65), actual revenue impact may be 30-50% higher.

WHAT THIS MEANS

Here's what we're seeing: A 8.7% lift in wRVU typically means your documentation is now capturing clinical complexity that was always there—providers aren't doing more, the notes are just reflecting reality better. Your 9% lift is above our typical 3-7% range, which is interesting. This could mean several things: your baseline documentation may have been particularly sparse, or you have complex patient populations that benefit more from thorough capture. It's worth exploring—if you can validate the baseline methodology, this becomes a powerful proof point.

●

TIME IN NOTES

Documentation efficiency

4.0 min/encounter  
saved per encounter

BEFORE ABRIDGE

10.00  
min/encounter

→

AFTER ABRIDGE

6.00  
min/encounter

=

+4.00  
40% reduction

BENCHMARK COMPARISON

Typical range: 3-5 min reduction

● Within typical range

WHAT THIS MEANS

Let's think about what 4 minutes saved really means: That's 2,383 hours annually—time that was previously locked up in documentation. The interesting question is: where does that time actually go? We've seen organizations channel it three ways: back to patients (more access, longer visits), back to providers (better work-life balance), or to operations (overtime reduction). Which path makes sense for you depends on your current priorities. The 4-minute reduction you're seeing is meaningful—it's the kind of lift that providers notice in their daily rhythm.

**SAME-DAY CHART CLOSURE**  
Documentation timeliness



**BENCHMARK COMPARISON**  
Typical range: 10-20 percentage point improvement

● Below typical

**WHAT THIS MEANS**

Chart closure is one of those metrics that tells a bigger story. When charts close same-day, it usually means providers are documenting in real-time rather than batching at the end of the day (or week). Your 5 percentage point improvement suggests the workflow is clicking—providers are trusting the AI draft enough to finalize notes between patients. Beyond the workflow signal, there's a downstream revenue impact: faster closure means faster billing cycles and reduced compliance exposure. Worth watching as your deployment matures.

💡 **EARLY INDICATORS**

🟢 **WHAT'S WORKING WELL**

- Revenue capture is responding—9% wRVU lift suggests documentation is better reflecting clinical complexity
- Providers are reclaiming time—4 min/encounter freed up, which compounds across your volume
- Real-time documentation emerging—5pt closure improvement suggests workflow is clicking

🟡 **AREAS TO WATCH**

- Utilization at 65%—worth exploring what's keeping remaining providers from adopting. Training gaps? Specialty-specific barriers?
- wRVU lift above typical—exciting if validated. Consider auditing baseline methodology to confirm this is real lift vs. measurement change
- Time savings currently unrealized—the hours are there, but haven't yet converted to patient access or reduced overtime. Worth a conversation with ops.

↗️ **OPTIMIZATION OPPORTUNITIES**

**1 DEEPEN ADOPTION**

**+\$25K/yr**

CURRENT

**65% of providers using consistently**

→

TARGET

**80-85% (where value compounds)**

RECOMMENDED ACTION

Look for patterns: Which specialties or sites are lagging? Often there are 2-3 common barriers that, once addressed, unlock the next wave of adoption.

**2 CONVERT TIME TO VALUE**

**+\$54K/yr**

CURRENT

**2,383 hrs/year reclaimed but not monetized**

→

TARGET

**15-20% converting to patient access or OT reduction**

RECOMMENDED ACTION

This is a collaboration opportunity with scheduling/ops. The time savings are real—the question is how to redirect that capacity intentionally rather than letting it dissipate.

**3 VALIDATE YOUR STANDOUT RESULTS**

**+\$0/yr**

CURRENT

**9% wRVU lift (above typical)**

→

TARGET

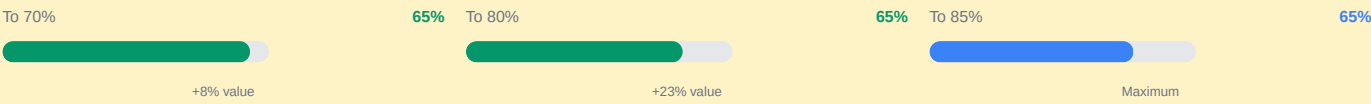
**Confirmed, defensible methodology**

RECOMMENDED ACTION

High-lift results are great news if they hold up to scrutiny. Quick audit: Was baseline measured consistently? Any other coding initiatives running in parallel? If you can answer "yes" to both, this becomes a compelling proof point.

💡 **EARLY STAGE CONTEXT**

At 65% utilization and 4 months in, you're still in early deployment. Here's the path forward:



📈 **PUTTING IT TOGETHER**

- \$118K documented value annually
  - Efficiency patterns emerging
  - Expansion potential: +\$383K
- Here's what's interesting: at 65% utilization, you're seeing this value with less than half your potential activated. That's not a criticism—it's actually good news. It means the math for scaling is compelling.

**KEY TAKEAWAY** The patterns here suggest 3 areas worth exploring. If you pursue all of them, the math points to an additional \$79K annually—but start with the one that feels most actionable for your organization.

THE PATH TO FULL SCALE

EXPANSION SUMMARY

Based on proven results at 50 providers and 65% utilization, scaling to 150 providers at 80% unlocks \$501K annually—an additional \$383K beyond current.

\$501K

PROJECTED ANNUAL VALUE

At full scale

+\$383K

ADDITIONAL VALUE

Beyond current

150

TARGET PROVIDERS

80% utilization

SCALING MATH

|                                       |        |
|---------------------------------------|--------|
| Current annual value                  | \$118K |
| × Provider scale (50 → 150)           | 3.0x   |
| × Utilization improvement (65% → 80%) | 1.23x  |
| Projected annual value at scale       | \$501K |

CURRENT VS. PROJECTED

CURRENT STATE

Providers: 50

Utilization: 65%

Annual value: \$118K

Value/provider: \$2K

➔

FULL SCALE TARGET

Providers: 150

Utilization: 80%

Annual value: \$501K

Added value: +\$383K

Here's how we think about this: What you're seeing today at 50 providers and 65% utilization isn't just a number—it's a proof point. The \$118K in annual value demonstrates the model works in your environment. The expansion math follows naturally: scale to 150 providers at 80% utilization, and you're looking at \$501K annually. The pattern is proven. Now it's about execution.

**KEY TAKEAWAY** Here's the math to keep in mind: every additional provider at target utilization adds roughly \$3K annually. You've proven the model works in your environment—the expansion opportunity is about replicating what's already working.

HOW WE CALCULATED YOUR VALUE

Let's walk through how we arrived at these numbers. Every calculation traces back to either your data or industry-standard benchmarks—we don't use optimistic assumptions. In fact, we lean conservative throughout, which means actual value is often higher than what we show here. Here's the methodology behind each component, so you can validate the logic yourself.

YOUR INPUTS

- 50 providers
- 55,000 annual encounters
- 65% utilization rate
- 4 months on Abridge
- 35,750 documented encounters

VALUE ATTRIBUTION

- wRVU: \$33/wRVU (Medicare), 50% attribution
- Time: \$150/hr provider value
- Retention: \$500K per departure
- Commercial payers: 30-50% higher

BENCHMARK RANGES

TYPICAL ABRIDGE RESULTS

- wRVU lift: 3-7%
- Time reduction: 3-5 min/enc
- Chart closure: +5-15 pp

QUALITY OF LIFE

- Pajama time: -2-5 hrs/wk
- Satisfaction: +10-20 pts
- Burnout: varies by org

WHAT WE DON'T INCLUDE

- Downstream revenue from patient experience
- Quality measure incentives (MIPS, HEDIS)
- Reduced compliance/audit risk
- Training time reduction & career satisfaction

Questions about our methodology? We're happy to walk through any calculation in detail. All assumptions are designed to be conservative and defensible.

**KEY TAKEAWAY** This analysis uses conservative assumptions throughout. With commercial payer rates and full attribution, actual value could be 30-50% higher than shown.